15. THE HEART : TAI CHI REMINDER & PRACTICE

DURATION : 03:38 | STUDY SHEET

COURSE SUMMARY

This video teaches how the heart interacts with the brain and the circulatory system through embryonic movements, while reviewing the theoretical bases of the heart-brain system. You will learn specific palpatory techniques to assess the heart, liver, and kidneys, as well as their emotional impact, particularly how the kidney is linked to fear. By integrating these concepts with pulsology, you will develop a holistic therapeutic approach, enabling you to support the body in its healing journey.

THE HEART: TAI CHI RECALL & PRACTICE

INTRODUCTION TO THE HEART-BRAIN SYSTEM

We will explore the intrinsic link between the **brain**, the **heart**, and the **circulatory system**.

The **mesenteric system**, as the axis of rotation of peritoneal development, performs a downward movement. This movement of the brain is synchronised with the movements of the **kidneys** and the **liver**.

We will re-examine the **motility** of brain development and integrate it with our cardiac knowledge, to understand how the heart supports the brain.

Remember that the heart can be positioned in the **vitelline vesicle** from the earliest stages.

PALPATION ASSESSMENT: LANDMARKS AND OBSERVATIONS

To begin, I take a precise landmark: the **angle of Louis**. I know it is located at the level of my **second intercostal space**.

I place my middle finger on the angle of Louis, then I place my hand and align it with the axis of the heart.

CARDIAC OBSERVATION

Facing me, the heart is in **pronation**, which corresponds to **diastole**. This indicates that it is seeking, searching.

In the engagement of its life, it is somewhat in renunciation, more withdrawn than in its action.

HEPATIC ASSESSMENT

I then place my hand at the hepatic level, with very light pressure, as if an **ant** were placed there that one could not crush.

I observe the liver. If it is rather horizontal, it may mean that it is slightly **swollen**.

It tends to move in this direction.

RENAL ASSESSMENT

Now, let's examine the kidneys. I palpate a kidney and observe that it is also in pronation.

This indicates that it is more prone to its lesion, which means that it is the kidney that destabilises it here, while the liver provides support.

The kidney is emotionally linked to **fear**. The patient is looking for a point of support, but their history is blocked at this level.

For this patient, I will therefore work on the kidneys to support their heart.

EMBRYONIC MOVEMENTS AND THERAPEUTIC SUPPORT

I remind you that the **embryonic movement** of the lungs is distinct.

While that of the heart is different. Observe the difference in these movements.

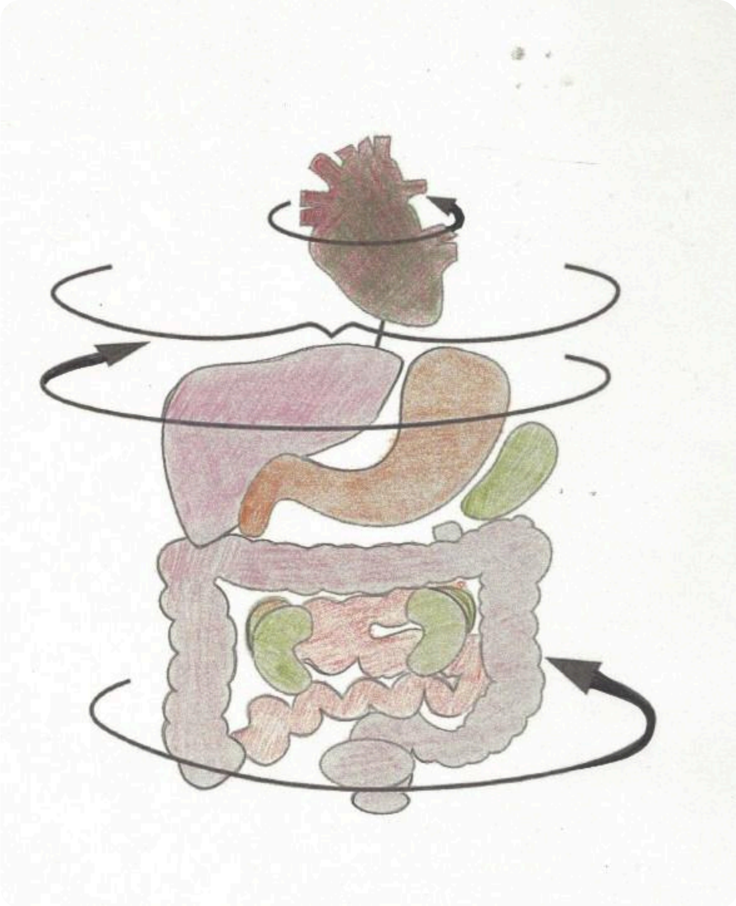

FIG. — Organ rotation

INDUCTION AND SUPPORT

After performing an induction towards its pronation, i.e., towards diastole, I support its kidneys.

Observe what happens. Do you feel the difference? The body seeks to reorganise itself.

Later, we will explore the motility of the development of the liver and kidneys, as well as their link with the **vascular system**.

THE HEART AND PULSOLOGY

It is essential to remember that the heart, in its form, is the reflection of the echo of its periphery.

This is why **pulsology** is such a fascinating field. It allows us to distinguish:

- A pure arterial type **pulse**.
- A venous type pulse, accompanied by **stases**.

For me, there are stases, probably located in the hepatic area.

An arterial pulse, when you take it, is characteristic, while a venous pulse manifests differently.

Despite this sensation on an artery, the question remains: am I more in **systole** or in **diastole** ultimately?